

Cataract and capsule laser in the uveitic eye

Part 2 | Decide what limits the visual potential

Cataract removal cannot resolve every uveitic cause of blur.

Preoperative finding	Implication
Lens opacity	Potentially reversible media limitation
Macular edema or scarring	Separate treatment or prognosis
Glaucomatous damage	Limits expected functional recovery
Synechiae and capsular support	Changes operative planning

Assess the macula when imaging is possible and explain uncertainty when the lens blocks the view. Elective surgery is usually planned after stable inflammatory control, with a perioperative strategy agreed in advance. An urgent indication may justify a different balance; no single calendar interval fits every disease phenotype.

Control and preparation are related but different

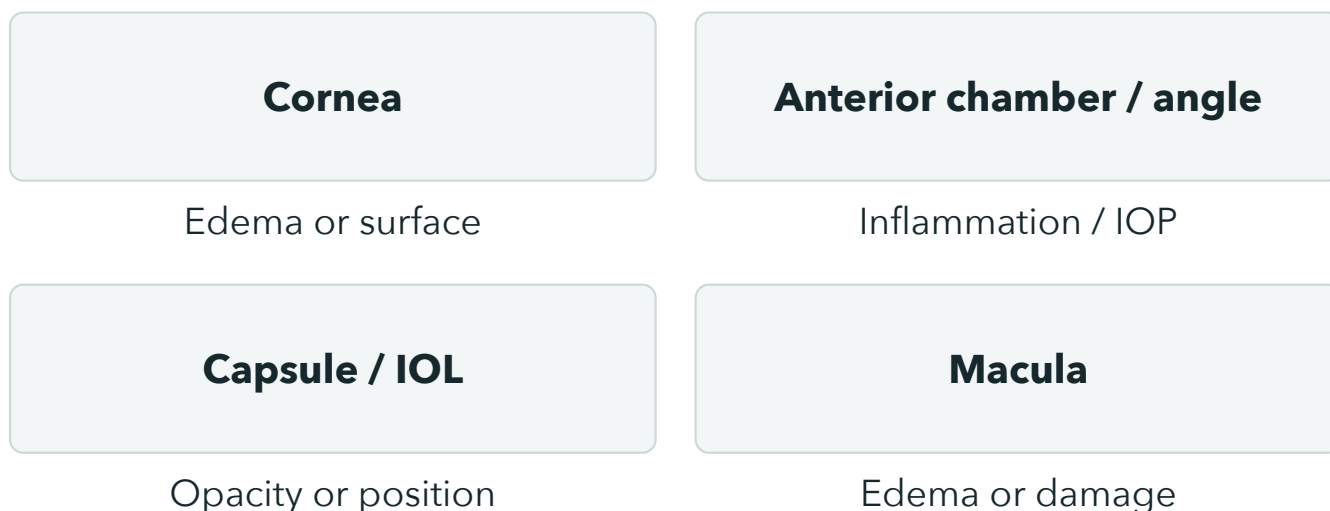
A quiet clinic visit does not replace a perioperative plan.

- Confirm sustained disease control and review prior postoperative flares.
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- Assess pupil, zonules, capsule, cornea and posterior segment.
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- Agree anti-inflammatory coverage and pressure surveillance.
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- Explain cataract, macular and glaucoma contributions to the outcome.

Complex anatomy may require pupil or capsular support and can increase postoperative inflammation. IOL suitability depends on age, disease phenotype, support and expected rehabilitation. Avoid promising that one IOL material eliminates inflammation or that lens extraction alone will restore a normal macula.

Postoperative blur requires compartment-based review

Opacity, inflammation, pressure and macular disease can coexist.



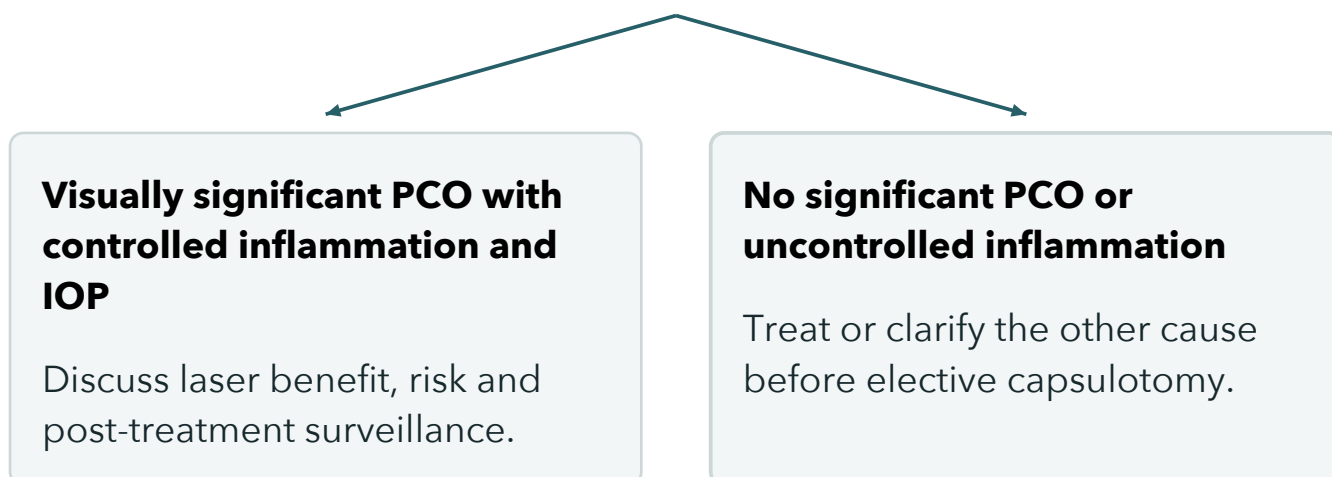
Original assessment map: each compartment supplies a different explanation for visual loss.

Recheck refraction, corneal clarity, IOP, anterior activity, capsule and macula. OCT can detect edema despite a modest anterior cell count. Treating pressure should not mean abruptly abandoning necessary inflammatory control, and adding steroid should not conceal a damaging pressure response. The treatment plan must address both processes.

Capsulotomy needs a demonstrated target

A clear capsule or active macular problem weakens the rationale for laser.

Is PCO the main reversible visual limitation?



A uveitic eye can develop pressure elevation, recurrent inflammation or macular edema after laser. Energy, retinal status and prior disease matter. Capsule opening is irreversible and may complicate later IOL exchange; it should not be used as an undirected diagnostic trial for persistent postoperative symptoms.

Read the evidence without overpromising safety

Small retrospective series cannot erase a clinically relevant complication risk.

Evidence observation	Interpretation
One small series reports no complications	Does not prove zero risk
Another series detects IOP and macular events	Population and technique may differ
Eyes selected for controlled inflammation	Do not extrapolate to active disease
Vision improves only in some eyes	Pre-existing pathology still limits benefit

Explain the expected benefit from removing genuine PCO and the residual visual limitations. Different retrospective cohorts may report different event rates without being logically incompatible. Use the patient phenotype and a monitoring plan rather than quoting a single percentage as a universal prediction.